

SUMMARY

Decision No. 419/05

17-Jun-2005

B. Alexander - B. Young - D. Beattie

- Continuing entitlement
- Aggravation (preexisting condition)

On April 5, 1993, the worker injured her back in a slip and fall while walking to the employer's store from the food court. On April 21, 1993, the worker had a non-compensable slip and fall.

The worker appealed the ARO's decision that denied her FEL and NEL awards, and granted lost time benefits from the date of her first accident to the date of her first return to work.

The worker was diagnosed with an acute exacerbation of her pre-existing condition and was unable to perform her previous tasks. Medical evidence satisfied the Panel that the worker's April 5, 1993 fall significantly contributed to her ongoing disability and prevented her from returning to work. Based on medical opinions and the worker's limited education and advanced age, the Panel was satisfied that the worker had been unemployable since she laid off work in 1993.

The worker was entitled to ongoing benefits, including total temporary disability benefits from when she was off work on April 21, 1993 to her return to work on May 3, 1993 and from May 18, 1993. She was also entitled to FEL awards and a NEL award for her permanent back impairment.

The appeal was allowed.

9 Page(s)

References: Act Citation

- WCA

Other Case Reference

- [w3605s]v

Style of Cause:

Neutral Citation: 2005 ONWSIAT 1364

WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 419/05

- [1] This appeal was heard in London on February 23, 2005, by a Tribunal Panel consisting of :
- B. Alexander : Vice-Chair,
B.M. Young : Member representative of employers,
D.B. Beattie : Member representative of workers.

THE APPEAL PROCEEDINGS

- [2] The worker appeals the decision of the Appeals Resolution Officer (ARO) dated August 21, 2002.
- [3] The worker appeared and was represented by Mr. D. Williams, a lawyer with the firm of Harrison Pensa. The employer is no longer in business.

THE RECORD

- [4] The Panel considered the material included in the Case Record prepared by the Office of the Vice-Chair Registrar (Exhibit #1) along with:
- Exhibit #2: Addendum #1 to the Case Record dated October 8, 2003;
Exhibit #3: Addendum #2 to the Case Record dated July 26, 2004;
Exhibit #4: the Hearing Ready Letter dated July 30, 2004; and
Exhibit #5: Addendum #3 to the Case Record dated February 21, 2005.
- [5] The worker and the worker's husband were in attendance. Mr. Williams made submissions.

THE ISSUES

- [6] The issue is whether the worker is entitled to further lost time benefits, future economic loss (FEL) awards and a non-economic loss (NEL) award, as a result of a compensable injury on April 5, 1993.

THE REASONS

(i) Background

- [7] The worker was born on March 24, 1943 and has a grade 10 education. She has worked in assembly line jobs, as a nurse's aide in a senior's home and in retail sales.
- [8] During her seven years as a nurses' aide, the worker sustained a number of compensable injuries to her back. Some of these injuries resulted in no lost time claims and others resulted in claims for which lost time benefits were paid.

[9] As a result of these claims, the worker's employer at the time applied for relief from the Second Injury and Enhancement Fund (SIEF) and in a decision dated December 21, 1983, denying such relief, a Board Review Specialist stated:

The Medical Adviser noted that from the medical reports there is no evidence of an underlying condition causing the injured worker to have recurring back problems. Each claim was instituted for a specific accident which occurred at work and the lost time involved for claim [sic] was minimal.

[10] On February 11, 1983, sustained another compensable injury for which she was granted a 10% permanent impairment award for her neck and shoulders. The assessor, Dr. P. Hopper, stated "This is a significant disablement in that it interferes with lifting, pushing, carrying."

[11] Because of her restrictions, the worker decided to move to a job with fewer physical demands and began a career in retail sales.

[12] The worker stated while she worked in sales, she continued to have back symptoms a few times every year. The pain would last for a couple of days and she would treat it with rest, heat and medication. She had epidural injections in 1991, but they didn't help.

[13] The worker joined the employer in October 1992, as manager of a retail clothing store in a shopping mall. By that time she had almost ten years of retail sales experience.

[14] On April 5, 1993, the worker injured her back in a slip and fall while walking back to the employer's store from a food court in the mall where she had purchased her evening meal.

[15] According to the worker, by the time she got back to the store, the pain was worse and she was in a great deal of discomfort. She called another employee to take her place and she laid off work. This was the first time since 1991 that she lost time from work because of back pain.

[16] The worker stated that when she got home she took Tylenol used a heat pad and went to bed. She felt worse the next day and she continued to rest at home and take Tylenol. She stayed at home for ten days before seeing her family doctor, Dr. G. Nancekievill, who had been her family doctor since the late 1970's. In the past when she experienced bouts of disabling back pain she was able to return to work after a few days of bed rest without seeing him.

[17] On April 15, 1993, Dr. Nancekievill noted that the worker had tenderness in her sacroiliac joints and over the coccyx as a result of falling on her back on April 5, 1993, and diagnosed "an acute exacerbation of a pre-existing back condition."

[18] The worker returned to work on April 19, 1993, and confined herself to light duties, doing paper work and supervising staff. Because of her back pain she wasn't able to do any arranging of stock or displays, or waiting on customers.

[19] On April 21, 1993, while making a purchase in a store in the mall on her way home at the end of her shift, the worker had a non-compensable slip and fall, further injuring her back. She was off work until May 3, 1993, when she returned to work, again to light duties.

[20] After a conversation with the employer in which it was agreed that she could not do the job she was hired for, she left work on May 18, 1993. She received nothing in writing from the employer, but was given to understand that they would have a job for her when she felt better. She has not returned to work of any kind.

[21] On May 18, 1993, Dr. Nancekievill reported that the worker had a “severe back disability” and would be off work indefinitely.

[22] Dr. Nancekievill further reported on September 27, 1993, as follows:

[The worker] experienced two separate falls at Masonville Mall on April 5, 1993 and on April 21, 1993. Both falls resulted in pain in her lower back. I first examined her on April 15, 1993 and she had tenderness in both sacroiliac joints and over her coccyx. She also had tender paravertebral muscle spasm in the lumbar area. And she had pain with flexion of her lumbar spine.

I saw her again on April 26, 1993, after her second fall and she was complaining of back and leg pain. On examination, she had tender sacroiliac joints, tender paravertebral muscle spasm in the lumbar area and pain with lumbar spine flexion. She was unable to work as manager of a dress shop at Masonville Mall.

Treatments included rest and Orudis SR 200 mg. daily on May 18, 1993 [the worker] required Demerol 50 mg. every four hours for pain. She was seen again on May 31 and June 14, 1993. Arrangements were made on June 24, 1993 for her to see Dr. Bellamy and he admitted her to Victoria Hospital. A CAT scan was done in Victoria Hospital and it showed bulging intervertebral lumbar discs. Arrangements were made for physiotherapy three times weekly and she was referred to the pain clinic for back injections. Dr. Bellamy started her on Nortriptylline 25 mg. at bedtime and Talwin 50 mg. as needed for pain.

She was last seen on September 16, 1993 and she was still suffering from back pain going down her left leg. On examination she had paravertebral muscle spasm, decreased flexion of her lumbar spine and positive straight leg raising of her left leg which showed that she had left sided sciatica.

The condition of her back prevents her from working and it will be several months before her prognosis will be known.

[23] Dr. Nancekievill reported on May 30, 1995, in part, as follows:

“[The worker] continued to have back pain and pain down her left leg and decreased mobility due to the injuries suffered on April 5, 1993 and April 21, 1993. She has seen a rheumatologist, Dr. N. Bellamy, who admitted her to Victoria Hospital and then her referred her to the Pain Clinic at Victoria Hospital. Injections in her for pain control helped some but the relief was minimal.

I continue to see her every four to six weeks and her condition [sic] and analgesics have not been very effective.

It has been over two years since her accident and therefore the prognosis for improvement is very slight.

[24] The worker sued the mall owner for damages in connection with the April 5, 1993 slip and fall and the mall owner contended that the worker’s cause of action was barred by the *Workplace Safety and Insurance Act, 1997* (WSIA), because her injury occurred while she was involved in an activity necessarily incidental to her employment.

[25] In a Tribunal decision, the Vice Chair found that the worker's injury arose in the course of her employment and was compensable under the WSIA. The worker could not therefore recover damages from the mall owner.

[26] The worker also sued the tenant, whose store she was in when she had her second fall.

[27] On February 18, 1999, Dr. S. Bailey, an orthopaedic surgeon, reported in part, as follows:

Specific examination of the lumbar spine noted a lordosis of approximately 15°. Flexion was limited to 10 to 15° due to discomfort. Extension was more comfortable and was able to be carried out to about 50% normal. Straight leg raising, I felt, was positive on the left at about 70 to 75° with back and buttock pain, whereas straight leg raising on the right was normal. Neurological examination of the lower extremities was normal.

Lumbar spine x-rays done today noted disc degeneration to be present at the L5 - S1 level, with disc space narrowing and end plate sclerosis. The remaining disc spaces appeared to be normal. There appeared to be no other specific bone abnormality. There were no findings suggestive of previous fracture.

...

The initial fall on April 5, 1993 was associated with a twisting, impaction injury resulted in further enhancement of symptoms within the low back. It is noted that the patient fell onto her right knee and then backwards onto her buttocks. I would suspect that the axial load applied to the lumbar spine was somewhat avoided by landing directly on the right knee prior to landing on the buttock/coccyx area. It appears that pre-existing symptoms were enhanced as a result of this injury. The patient had to remain off work, but had improved somewhat to a point to allow her to return to work.

At the time of the second fall, [the worker] appeared to be still suffering from enhanced symptoms within the low back and left leg. The second fall initiated further axial compression to the lower lumbar spine and a further enhancement of symptoms. Because there appeared to be no structural damage sustained, as verified on CT scan and x-rays, the injuries would be more interpreted as soft tissue in nature. Symptoms from these soft tissue injuries would improve, as expected healing occurred. The time line for this to happen is difficult to establish, but it appears that over the subsequent two to three years, the symptoms have continued to improve slowly. This would be as one would expect, considering the nature of her injuries super-imposed on pre-existing symptomatic degenerative changes.

Certainly, [the worker] was experiencing significant discomfort in the low back with radiation into the right leg prior to her two subsequent falls. This required ongoing medical care, including multiple needle injections. Indeed, her symptoms were classified as being chronic and were thought to be related to the underlying degenerative disc disease that was present. The falls that occurred in April 1993 obviously enhanced the symptoms related to this condition. The initial fall of April 5, 1993 precipitated further symptoms and discomfort, and this was once again enhanced by her second fall of April 21, 1993. The exact contribution to the enhancement of her previous chronic complaints by each fall is very difficult to discern, however, analyzing the mechanism of injury, as well as the subsequent complaints, I would feel that both falls contributed to a similar enhancement of symptoms, i.e. 50% each. I do not feel that one was more responsible than the other for the persistence of symptoms from which she apparently complains.

...

I would feel that it would be unlikely that she could continue to carry on with active full - time employment on a competitive basis because of these limitations. I would feel, however, that the majority of her symptoms are related to the degenerative disc disease

and subsequent symptoms that have been present for an extended period of time. Some of the symptoms she is experiencing now within the low back can be somewhat attributed to the injuries sustained in the two falls in April 1993. The specific amount of enhancement is difficult to denote, however, I would feel that it would be in the area of 25 to 30%. No doubt, if the patient had not sustained an injury to her low back, her chronic symptoms would have followed a natural course with increasing discomfort and subsequent further limitations of activities. I would find it difficult to predict whether she would have been unable to be employed as a consequence of these expected increased complaints if they had not been enhanced by the injuries sustained at the time of the falls.

....

It would be my opinion that these findings would not completely inhibit her from returning to active employment, although I would suggest that it would be on a part-time basis. She would have to have persistent limitations with regard to prolonged standing, repetitive lifting and bending, although light sales activity, I feel, could be tolerated on a part-time basis. This opinion is directly related to the findings within the low back.

Other musculoskeletal complaints that she is now experiencing, related to arthritis and fibromyalgia, would be sufficient, on a cumulative basis, to prevent her from returning to even part-time working activities.

- [28] The Tribunal decision was forwarded to the Board and on August 24, 2002, the ARO granted the worker lost time benefits from April 5, 1993, the date of her first slip and fall, to the date of her first return to work, April 19, 1993. The ARO denied the worker further benefits, including FEL and NEL awards, stating:

The continuing problems and employment loss are not the responsibility of this claim, but the second compensable accident, pre-existing condition and other non work related factors. I do not disagree that the worker was totally impaired, as she has qualified for Canada Disability benefits, but the responsibility is not the solely the result of this claim

- [29] The worker proceeded with her tort action against the Board in regard to her second slip and fall and the matter was settled on the basis that liability rested 50% with the Board and 50% with the tenant.

(ii) Mr. Williams submissions

- [30] Mr. Williams submitted that although the worker had pre-existing back problems, she was able to continue to work and cope with her problems.

- [31] Mr. Williams referred to Dr. Nancekievill's clinical notes dated June 16, 1992, June 26, 1992, October 8, 1992 and November 20, 1992, in which there was no reference to back pain and his note of December 14, 1992, which referenced back pain but not increased back pain, despite the fact that this was in the middle of the hectic Christmas season.

- [32] Mr. Williams submitted that Dr. Nancekievill's April 15, 1993 report indicated that the worker had new back pain symptoms and that the condition of her back had worsened as a result of her April 5, 1993 slip and fall.

- [33] In reference to Dr. Bailey's report, which stated that it would be unlikely that the worker would be able to carry on active full time employment on a competitive basis because of her limitations, Mr. Williams pointed out that at the time of her first slip and fall the worker was

working full time and although not free of symptoms was competitively employed and able to function at home and at work.

[34] Mr. Williams noted that when the worker did return to work after her April 5, 1993 fall, she returned to lighter duties as she was not able to carry out her usual tasks.

[35] Mr. Williams pointed out that, according to Dr. Bailey, the two falls contributed equally to the increase in the worker's symptoms.

[36] According to Mr. Williams, the result of the Tribunal decision was that the worker was prevented from recovering significant damages as a result of the first fall and although the action regarding the second fall was settled on the basis that it was agreed and understood that liability for the results of both the first and the second fall rested 50% with the Board, the worker has received nothing from the Board.

(ii) Relevant law and policy

[37] Since the worker was injured on April 5, 1993, the pre-1997 *Workers' Compensation Act* is applicable to this appeal. The hearing of this appeal commenced after January 1, 1998; therefore, certain provisions of the *Workplace Safety and Insurance Act, 1997* ("WSIA") also apply to the appeal. All statutory references in this decision are to the pre-1997 Act, as amended, unless otherwise stated.

[38] Pursuant to s.126 of the WSIA, the Board stated that the following policy packages are applicable to this appeal:

Package #35 – Continuing Entitlement/NEL – DOA from January 2, 1990 to December 31, 1997

Package #66 - FEL Decision – as of January 1, 1998,

Package #87 – Non-Work Related Conditions

Package #107 - Aggravation Basis or SIEF

Package #300 - Decision Making/Benefit of Doubt/Merits and Justice.

[39] We have considered these policies as necessary in deciding the issues in this appeal.

(iii) The Panel's reasons

[40] In order to find entitlement to benefits, a workplace injury must have made a significant contribution or material contribution to a worker's ongoing disability. The issue for the Panel is whether the worker's first 1993 fall contributed in a significant or material way to the worker's subsequent impairment.

[41] Dr. Bailey offered the opinion that the majority of the worker's symptoms related to the worker's degenerative disc disease and fibromyalgia and that the contribution of the worker's two falls to her ongoing condition was between 25% and 30%.

- [42] According to the Board's December 21, 1983 report, prior to working as a nurse's aide in seniors home, there was no evidence that the worker had any back problems.
- [43] Subsequent to her February 11, 1983 neck and shoulder injury and despite the back problems she developed as a result of her numerous compensable back injuries while working as a nurse aide, the worker was able to successfully change careers and work in the physically less demanding world of retail sales for a period of almost ten years.
- [44] The worker was able to work for the employer as a store manager without losing any time from work from October 1992 until her April 5, 1993 fall and according to Dr. Nancekievill's notes during that period complained on only one occasion about back pain.
- [45] The Panel is satisfied that the worker's April 5, 1993 injury was significant. Dr. Nancekievill diagnosed an 'acute exacerbation' of the worker's pre-existing condition and as a result of this fall and when she returned to work, the worker was unable to perform the tasks she had previously been able to perform. Dr. Bailey found that the April 5, 1993 fall was equally responsible for the enhancement in the worker's symptoms as the worker's April 21, 1993 fall.
- [46] Although Dr. Bailey stated the worker's ongoing symptoms were more related to the worker's underlying conditions, than her 1993 falls, the evidence shows that as a result of her first 1993 fall, the worker sustained a sufficiently severe aggravation of her symptoms that combined with the second fall, she was unable to return to the work that she had been successfully performing for a period of ten years.
- [47] Dr. Bailey has indicated that the worker's 1993 falls are equally responsible for the "enhancement of symptoms" that prevented the worker from returning to work.
- [48] Based on the above, the Panel is satisfied that the worker's April 5, 1993 fall made a significant contribution to her ongoing disability.
- [49] The fact remains that had the worker not sustained the two falls, she would likely have been able to continue to work at the profession of retail sales, which she had worked at successfully for the ten years prior to the falls.
- [50] The Panel therefore finds that the worker has initial entitlement for a back injury as a result of a workplace accident on April 5, 1993 and has an ongoing permanent back impairment, for which Dr. Bailey provided range of motion restrictions.
- [51] The Panel is further satisfied that at the time of the worker's second fall, the worker was totally unable to work. Dr. Nancekievill expressed that opinion on May 18, 1993 and on September 27, 1993 and although in his May 30, 1995 report, Dr. Nancekievill did not comment on the worker's employability he advised that the worker's symptoms continued and that his prognosis for her improvement was "very slight."
- [52] In his February 18, 1999 report, Dr. Bailey advised that the worker would be unlikely to return to active employment.

- [53] Based on these medical opinions and the worker's limited education and advanced age, the Panel is satisfied, on a balance of probabilities, that the worker has been unemployable since she laid off work in 1993.

THE DECISION

- [54] The appeal is allowed.

- [55] The worker is entitled to ongoing benefits, including total temporary disability benefits for the period she was off work from April 21, 1993 until her return to work on May 3, 1993 and from May 18, 1993; as well as full FEL awards and a NEL award for her permanent back impairment.

DATED: This 17th day of June, 2005.

SIGNED: B. Alexander, B.M. Young, D.B. Beattie